A clearer path to discharge.

ClearPath — AMD-assisted discharge coordination

- One reviewable workflow for drafts, approvals, and findings.
- Synthetic-data prototype · Human approval remains explicit.

A change should not leave approval behind.

Built for the discharge coordinator

- Clinical documentation, pharmacy, and billing depend on shared information.
- A corrected draft can make earlier approvals and findings outdated.
- Coordinators need to see what is current and what still needs review.

Review once. Check together.

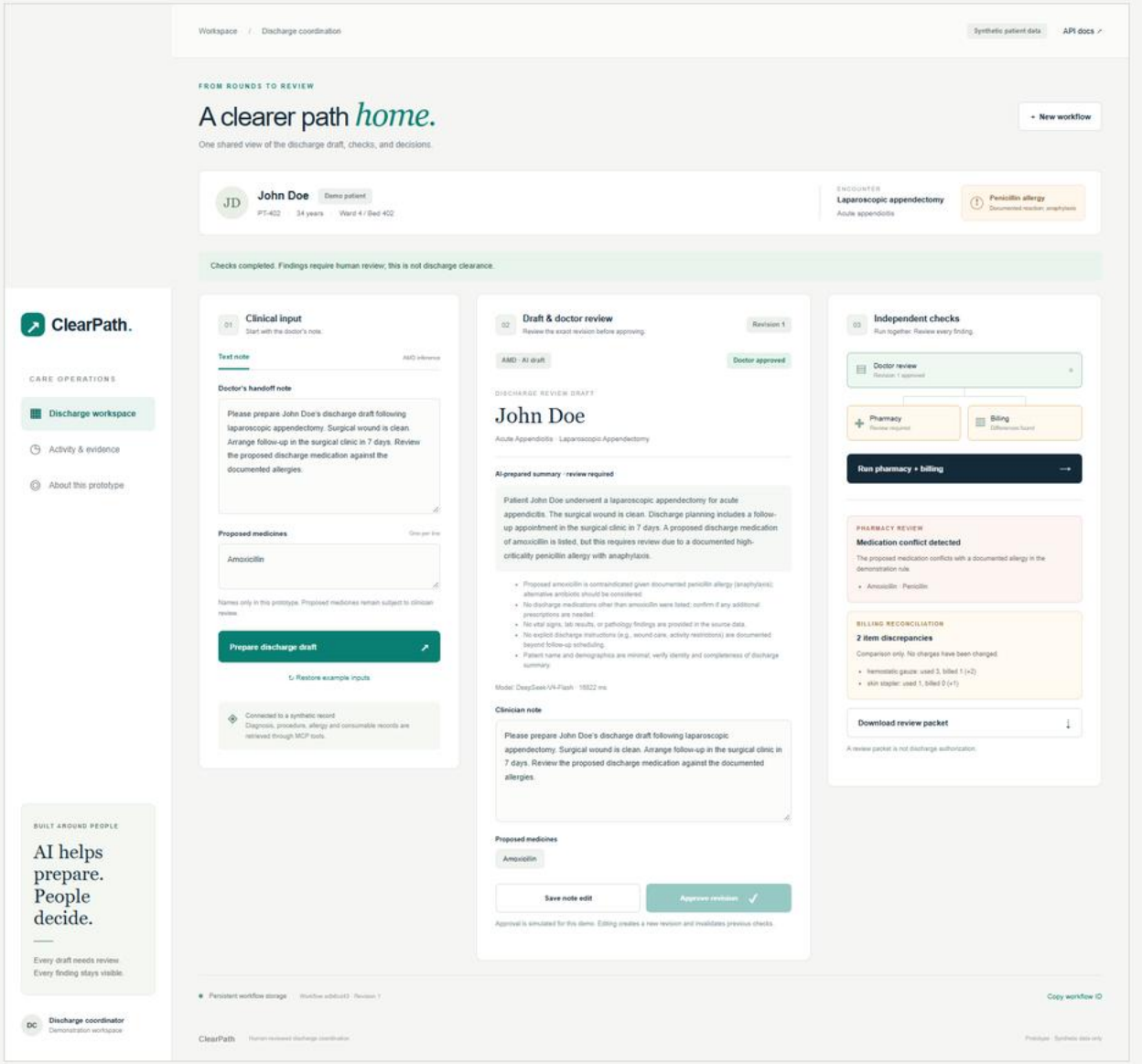
An explicit sequence with visible outcomes

- 1 Generate a draft with DeepSeek-V4-Flash through the configured AMD API.
- 2 Approve the specific revision under review.
- 3 Run independent pharmacy and billing checks concurrently.
- 4 Return findings for human review; persist the workflow.

A real workflow. Visible findings.

Actual AMD-assisted run
Synthetic patient

- AMD-assisted draft reviewed before independent checks.
- Deterministic MCP tools return pharmacy and billing findings.
- Human-review packet. No autonomous discharge clearance.



Every result belongs to a revision.

Traceable state changes are the core of the prototype

- Edits invalidate the previous approval and stored findings.
- Revision checks reject results that became stale while workers ran.
- SQLite restores saved snapshots after restart.
- Interrupted work becomes a visible failure and can be retried.

AMD drafts.
Rules check records.

DeepSeek-V4-Flash → human review → deterministic MCP checks

- A live configured AMD endpoint call was verified using synthetic data.
- FastAPI + Pydantic: validated requests and structured results.
- FastMCP + asyncio: concurrent pharmacy and billing rule checks.
- SQLite: persist the latest workflow snapshot and its revision.

Coordinate first.
Validate before scale.

A working foundation with an honest scope

- Today: AMD-assisted drafting, synthetic records, simulated approval.
- Next: authenticated reviewers and validated EHR / insurer integrations.
- Next: clinical evaluation, multimodal inputs, and a complete audit history.
- A successful endpoint call demonstrates integration, not clinical validation.